AE Intake Readiness Checklist

A working list for the source in front of you, and for the case data to have before anyone prepares an E2B(R3) file. This is not a standard operating procedure.

Intake before the case is in the safety database

- ☐ The source is the document received: email, PDF, call note, CIOMS, or MedWatch. It is not a retelling of that document.
- ☐ Day 0 is the date the sponsor, or the CRO acting for the sponsor, first had the report. A later data-entry date does not reset it.
- ☐ Reporter name or role is recorded, and whether the source says they are a health professional.
- ☐ The patient identifier is the one the procedure uses: subject ID, initials, age, sex. Do not invent a field the source does not have.
- ☐ Suspect product and dose are recorded. Lot or batch is recorded when this is also a product quality complaint.
- ☐ The event is written in the reporter’s words, and then as the term a reviewer will code.
- ☐ Seriousness is checked against the ICH list: death, life-threatening, inpatient hospitalization or prolongation, persistent or significant disability, congenital anomaly, or a medically important event. A laboratory grade alone is not that list.
- ☐ A named person accepts or overrides the draft. The software does not decide seriousness, causality, or that a case is a SUSAR.
- ☐ AE Connect integrates the case with any safety database, including Oracle Argus, ArisGlobal LifeSphere, Veeva Vault Safety, and others. Those products remain the system of record.

Data to have before anyone prepares an E2B(R3) file

AE Connect integrates the case with any safety database. Oracle Argus, ArisGlobal LifeSphere, and Veeva Vault Safety remain the system of record for the E2B(R3) file.

- ☐ The worldwide unique case identifier is assigned in the safety database, which remains the system of record. It is not assigned on this checklist.
- ☐ Sender and receiver are the safety database's trading partners. That database remains the system of record for them.
- ☐ Patient information is limited to what the narrative actually supports.
- ☐ The reaction is kept verbatim, plus a MedDRA term a coder will confirm. AE Connect can suggest a preferred term. A person confirms it.
- ☐ The drug line has the suspect product, the dose, dates if they are known, and the action taken if the source says so.
- ☐ Causality is the reviewer's call. WHO-UMC words (certain, probable, possible, unlikely, conditional, unassessable) are one way to write it down. Possible is not proof.
- ☐ Expectedness is checked against the Investigator's Brochure or the company core data sheet version named in the protocol. AE Connect does not hold that document. The draft only flags that someone should check it.
- ☐ The narrative is one the safety database can store.
- ☐ For a US IND, an unexpected fatal or life-threatening suspected adverse reaction is reported as soon as possible and no later than 7 calendar days after initial receipt. Other serious and unexpected suspected adverse reactions are reported as soon as possible and no later than 15 calendar days after the sponsor determines the information qualifies (21 CFR 312.32). The procedure cites the text you follow. This checklist is not that procedure.
- ☐ Still outside this PDF: the E2B(R3) file itself, the gateway, unblinding, the letter to investigators, and aggregate reports (DSUR, IND annual report, PSUR, PBRER).